

Actihist-Co PE Syrup (Sugar Free)

VIACTXX-PC2

DESCRIPTION

Red, transparent liquid with tutti frutti flavour.

COMPOSITIONS

Each 5 ml contains:

Tripolidine Hydrochloride	1.25 mg
Phenylephrine Hydrochloride	5 mg
Dextromethorphan Hydrobromide	10 mg

ACTIONS AND PHARMACOLOGY

- **Tripolidine:** Tripolidine is an antihistamine. It is an antagonist of the H-1 receptor effects of histamine. It acts by competing with histamine for H-1 receptor sites on effector cells. It diminishes or abolishes most of the actions of histamine mediated by the H-1 receptors. The action of histamine most readily affected is that on the involuntary muscle of the intestines, bladder, uterus and bronchioles.
- **Phenylephrine:** Phenylephrine is used as a nasal and bronchial decongestant. It is a direct selective alpha-adrenergic receptor agonist, and thus it does not cause the release of endogenous noradrenaline like pseudoephedrine does.
- **Dextromethorphan:** Dextromethorphan Hydrobromide is a cough suppressant which has a central action on the cough centre in the medulla. This medication is well absorbed from the gastrointestinal tract, is metabolised in the liver and undergoes renal excretion.

PHARMACOKINETICS

After absorption from the gastrointestinal tract, tripolidine is metabolized; a carboxylated derivative accounts for about half the dose excreted in the urine. Reported half-lives vary from 3 to 5 hours or more. Tripolidine is distributed into breast milk.

Phenylephrine has low oral bioavailability owing to irregular absorption and first-pass metabolism by monoamine oxidase in the gut and liver.

Dextromethorphan Hydrobromide is rapidly absorbed from the gastrointestinal tract. It is metabolized in the liver excreted in the urine as unchanged dextromethorphan and demethylated metabolites including dextrophan. It is reported to act within half an hour of administration by mouth and to exert an effect for up to 6 hours.

INDICATION

This medication is:

- indicated for the symptomatic relief of upper respiratory tract disorders which are benefited by a combination of a histamine H1-receptor antagonist, a nasal decongestant and an antitussive.
- relieve dry irritating coughs such as those associated with common cold, upper respiratory tract infections, allergic (seasonal) rhinitis, vasomotor (perennial) rhinitis and hay fever.
- help dry up running nose and open congested bronchi to aid clear breathing.

CONTRAINDICATIONS

Persistent chronic & productive cough, liver disease, asthma, concomitant or within 2 weeks of monoamine oxidase inhibitors (MAOI) therapy, severe hypertension (HTN) or severe coronary artery disease.

WARNINGS AND PRECAUTIONS

- Avoid alcohol.
- It should be carefully considered when the following medical problems exist: epilepsy, prostatic hypertrophy, glaucoma, hepatic disease, hypertension (HTN), heart disease, hyperthyroidism, peptic ulcer stenosis, pyloro-duodenal & bladder neck obstruction.
- This preparation may cause drowsiness. If affected do not drive motor vehicle or operate machine.
- Not to be used in children less than 2 years of age.
- To be used with caution and doctor's/ pharmacist's advice in children 2 to 6 years of age.

DRUG INTERACTIONS

Betahistine, anxiolytics & hypnotics, metoclopramide, antiparkinson drugs.

PREGNANCY AND LACTATION

As with any other drugs, use in pregnancy and lactation is best avoided.

ADVERSE EFFECTS

Drowsiness, dizziness & constipation.

OVERDOSE AND TREATMENT

Clinical features: respiratory depression, paranoid psychosis, delusions, hallucinations and convulsion.

Treatment: Treatment should include emptying the stomach by aspiration or gastric lavage. Nervous stimulation and convulsions should be treated with a sedative such as diazepam intramuscularly. If marked excitation is present, a sedative such as diazepam or a short-acting barbiturate may be given.

Severe overdose of phenylephrine may product hypertension and associated reflex bradycardia. Treatment measures include early gastric lavage and symptomatic and supportive measures. The hypertensive effects may be treated with an alpha-receptor blocking agent (such as phentolamine mesilate 6-10 mg) given intravenously, and the bradycardia treated with atropine, preferable only after the pressure has been controlled.

DOSAGE AND ADMINISTRATION

Oral

- Adult and children above 12 years: 10 ml, 3 times a day.
- Children 6 to 12 years: 5 ml, 3 times a day.
- Children 2 to 5 years: 2.5 ml, 3 times a day.

Note: The information given here is limited. For further information consult your doctor or pharmacist.

Storage:
Store below 30°C. Protect from light.

Presentation/Packing:
Syrup x 120 ml

Product Registration Holder /
Manufactured by: HOVID Bhd.
121, Jalan Tunku Abdul Rahman
(Jalan Kuala Kangsar),
30100 Ipoh, Malaysia.

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